PRACTICE EXAM 11 / EXAM SET 02

Domain 4: Patient Care Coordination and Education

12 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Preventive Care and Care Gaps

> **SCENARIO** During previsit planning, an assistant sees that the provider wants an updated cigarette-smoking history before discussing lung cancer screening. The chart says the patient currently smokes one pack daily, but the patient reports quitting several years ago and having smoked different amounts over time.

Which information should the assistant gather for the provider's screening review?

A. The patient's preferred imaging facility and whether morning appointments are convenient.
B. The amounts smoked during each period, the years of smoking, and when the patient quit.
C. The cigarette brand most recently used and whether family members prefer another brand.
D. The patient's willingness to schedule imaging before the provider reviews the smoking history.

Q02 | Patient Education and Self-Management Support

> **SCENARIO** A dietitian's written plan limits each snack to no more than 200 mg of sodium. The assistant reinforces how to apply that plan using sample Nutrition Facts labels. Each sodium amount below is listed per serving, and the patient will eat the full number of servings stated in the option.

Which snack amount fits the documented sodium limit?

A. Two servings of a snack containing 90 mg of sodium per serving.
B. Two servings of a snack containing 110 mg of sodium per serving.
C. One serving of a snack containing 250 mg of sodium per serving.
D. Three servings of a snack containing 80 mg of sodium per serving.

Q03 | Community Resources and Access Barriers

SCENARIO A patient says an ordered medication remains at the pharmacy because its cost is more than the patient can afford. The patient agrees to help from the clinic's medication-access navigator. The navigator can investigate assistance programs and communicate with the prescribing team about available options.

Which action best addresses this barrier within the assistant's role?

A. Suggest taking the medication less often so each prescription lasts longer.

B. Give a generic discount card and document that medication access is restored.

C. Connect the patient with the navigator and follow up on whether the prescribed medication becomes accessible.

D. Recommend a less expensive over-the-counter product in place of the prescription.

Q04 | Referrals and Care Transitions

SCENARIO A hospital discharge plan calls for a primary care visit within seven days. During coordination, the assistant finds that the next routine opening is in three weeks. Clinic policy directs assistants to send such timing conflicts to the designated follow-up nurse, who works with the provider to arrange an appropriate appointment.

How should the assistant handle this scheduling conflict?

A. Send the discharge deadline and availability problem to the follow-up nurse and track the resulting appointment arrangement.

B. Book the routine opening and document that any scheduled visit satisfies the discharge plan.

C. Ask the patient to use a walk-in clinic instead and close the primary care follow-up task.

D. Revise the documented follow-up window to three weeks to match routine availability.

Q05 | Preventive Care and Care Gaps

SCENARIO A laboratory rejects a mailed colorectal screening specimen because the collection container lacks the required patient label. No screening result is issued. The clinic's written recollection protocol authorizes the assistant to arrange a replacement kit, reinforce the laboratory's labeling instructions, and notify the ordering provider.

How should the screening task be handled?

A. Mark screening complete because the patient collected and mailed the specimen.

B. Keep screening unresolved and arrange recollection using the clinic's protocol.

C. Enter the rejected specimen as a negative screening result in the record.

D. Document that the patient declined screening and remove the task from tracking.

Q06 | Patient Education and Self-Management Support

SCENARIO A patient's provider-approved home plan says to call the clinic the same day for a weight increase of 2 lb or more in 24 hours. During a follow-up teaching call, the patient reports yesterday's weight as 148 lb and today's as 151 lb. The assistant asks the patient to explain how the written plan applies.

Which patient statement best demonstrates understanding of this part of the plan?

A. "I will wait until the increase has lasted a week before contacting the clinic."
B. "I will double my medication today because my weight has increased."
C. "I will record the weights and wait to mention the change at my next routine visit."
D. "My weight increased by 3 lb since yesterday, so I need to contact the clinic today."

Q07 | Community Resources and Access Barriers

SCENARIO A patient wants to attend an ordered virtual diabetes education visit but has no internet service or suitable device. A community partner offers private telehealth rooms with video equipment and technical assistance. The patient agrees to exploring this resource; no room has been reserved.

Which coordination action best supports access to the visit?

A. Send the video link again and ask the patient to open it before the visit.
B. Ask the educator to mark the visit completed after mailing a brochure.
C. Recommend using a public computer area without checking whether privacy is available.
D. Confirm a private room and equipment are available at the visit time and help the patient arrange access.

Q08 | Referrals and Care Transitions

SCENARIO A specialist's consultation report has arrived and been attached to the chart. It recommends that the primary care provider consider additional testing. The report has not been reviewed by that provider, and the referral task currently has no assigned follow-up owner. The clinic closes referral tasks only after provider review and documentation of the resulting follow-up plan.

Which action best advances this referral toward appropriate closure?

A. Schedule the suggested tests directly from the specialist's recommendation without seeking an order.
B. Close the referral task because the consultation report has been uploaded successfully.
C. Route the report for provider review and assign follow-up tracking through the clinic workflow.
D. Tell the patient to arrange the suggested tests and remove the task from the clinic queue.

Q09 | Patient Education and Self-Management Support

SCENARIO A patient with low vision cannot use the standard printed self-care handout and asks for an audio version. The clinic has a provider-approved audio version of the same instructions. The patient can operate the audio player independently and agrees to review the instructions with the assistant.

Which education approach best matches this patient's needs?

A. Give the standard handout again and ask the patient to read it under brighter light.
B. Provide the audio file and document understanding based on the successful file transfer.
C. Review the approved audio instructions with the patient and check understanding of the required actions.
D. Ask a family member to take over the teaching even though the patient prefers independent use.

Q10 | Community Resources and Access Barriers

SCENARIO An older adult reports receiving a home heating shutoff notice and requests help before service is interrupted. The patient reports no current symptoms or immediate exposure emergency. The clinic's social worker coordinates local utility-assistance resources, but eligibility and processing time must be checked.

Which action best addresses this nonclinical support need?

A. Give the patient the clinic appointment line and ask them to report symptoms if heating stops.
B. Connect the patient with the social worker, communicate the shutoff deadline, and track the assistance connection.
C. Assure the patient that the utility company will suspend the shutoff once assistance is requested.
D. File the notice in the record and wait until the next routine visit to discuss available programs.

Q11 | Preventive Care and Care Gaps

SCENARIO An assistant calls about an overdue screening mammogram. The patient reports a newly noticed breast lump and asks to use the existing screening appointment next month to have it checked. Clinic policy requires new breast concerns to be routed to the nurse the same day for clinical direction.

Which action appropriately handles this information?

A. Reassure the patient that the scheduled screening will address the lump and keep the current outreach plan.
B. Change the existing screening order to a diagnostic study without contacting the provider.
C. Recommend that the patient complete screening before reporting the symptom to the clinical team.
D. Document the new concern and arrange same-day nurse contact under clinic policy.

Q12 | Referrals and Care Transitions

> **SCENARIO** After discharge, a patient is following an ordered wound-care plan with home nursing. The nurse reports that the supply vendor delivered only part of the ordered dressing materials. The next home visit is tomorrow. Clinic policy directs supply interruptions to the coordinating nurse, with assistants authorized to contact vendors about existing orders.

Which action best supports continuity of the ordered care?

A. Notify the coordinating nurse and contact the vendor to resolve the missing ordered supplies before the planned visit.

B. Choose similar materials from an office catalog and advise the patient to substitute them.

C. Cancel tomorrow's home visit because part of the supply shipment has not arrived.

D. Tell the patient to wait for the vendor's next routine shipment before contacting the care team.

ANSWER KEY & EXPLANATIONS

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01 B	02 A	03 C	04 A	05 B	06 D
07 D	08 C	09 C	10 B	11 D	12 A

DETAILED EXPLANATIONS

Q01 | Correct answer: B

The amounts smoked during each period, the years of smoking, and when the patient quit.

A detailed exposure history and quit timing help the provider evaluate screening needs and correct outdated information. The assistant gathers and documents these facts without independently determining eligibility or ordering screening.

Q02 | Correct answer: A

Two servings of a snack containing 90 mg of sodium per serving.

Two servings multiplied by 90 mg per serving equals 180 mg, which meets the 200 mg snack limit. The other amounts provide 220 mg, 250 mg, and 240 mg, respectively; the per-serving value must be multiplied by the number of servings eaten.

Q03 | Correct answer: C

Connect the patient with the navigator and follow up on whether the prescribed medication becomes accessible.

The navigator offers a concrete route for investigating affordability while keeping medication decisions with the prescribing team. Giving a discount card does not establish that the patient can obtain the prescription, and the assistant should not change the regimen or substitute another product.

Q04 | Correct answer: A

Send the discharge deadline and availability problem to the follow-up nurse and track the resulting appointment arrangement.

The routine opening falls outside the documented transition plan, so the assistant should use the clinic's escalation pathway to obtain an authorized arrangement. Scheduling outside the window does not resolve the conflict, and the assistant should not revise the discharge plan.

Q05 | Correct answer: B

Keep screening unresolved and arrange recollection using the clinic's protocol.

A rejected specimen without a result does not complete the screening. The supplied protocol provides an authorized way to correct the collection problem while maintaining tracking and notifying the provider; neither a negative result nor refusal can be inferred.

Q06 | Correct answer: D

"My weight increased by 3 lb since yesterday, so I need to contact the clinic today."

The increase is 151 lb minus 148 lb, or 3 lb in 24 hours, which meets the call threshold in this patient's supplied plan. The statement demonstrates application of the instructions rather than simple recall; it does not authorize a medication change.

Q07 | Correct answer: D

Confirm a private room and equipment are available at the visit time and help the patient arrange access.

The barrier involves both technology and a usable setting, so the resource must be matched to the actual appointment time and privacy needs. A link or brochure alone does not provide access to the ordered interactive visit.

Q08 | Correct answer: C

Route the report for provider review and assign follow-up tracking through the clinic workflow.

Receipt of the report is one step, but the supplied closure workflow also requires provider review and a documented follow-up plan. The assistant can establish tracking responsibility and route the recommendation without turning it into an independent test order.

Q09 | Correct answer: C

Review the approved audio instructions with the patient and check understanding of the required actions.

The audio format addresses the stated access need while preserving the established clinical instructions and the patient's independence. Reviewing the content and checking understanding are still necessary; successful delivery of a file does not demonstrate learning.

Q10 | Correct answer: B

Connect the patient with the social worker, communicate the shutoff deadline, and track the assistance connection.

A specific resource connection that includes the deadline helps the social worker investigate a timely response. Assistance eligibility and processing time are not established, so the assistant should not promise that submitting a request will prevent interruption.

Q11 | Correct answer: D

Document the new concern and arrange same-day nurse contact under clinic policy.

The new symptom changes the coordination need and must be routed through the supplied clinical pathway. The assistant should not assume that routine screening will evaluate the concern, provide reassurance about the lump, or independently change the test order.

Q12 | Correct answer: A

Notify the coordinating nurse and contact the vendor to resolve the missing ordered supplies before the planned visit.

The interruption should be made visible to the responsible nurse while the assistant works on delivery of the existing order. Selecting replacement clinical materials or canceling planned care would change the plan without authorization; timely escalation allows the nurse to direct any necessary contingency.

End of Practice Exam